CenterWell Home Health

PREPARED FOR
CWHH CLINICAL QUALITY TEAM

Clinical Quality Intelligence

Which patient needs care today, before it's too late?

Weekly Patient Conference Summary

High-Risk & Rising-Risk Patients | Week of 08/17/2026

CQI is a framework for identifying risk, care gaps and opportunities to improve patient outcomes. It is **proactive**: it shows which patient needs care this week, why, and what to do **while there is still time to change the outcome**.

Patient

Patient ID	**9000123**	60-day risk	**High 0.38**
Episode	**7000456**	Status	**Current, day 36 of 60**
Branch	**Branch 0412**	CQI run	**08/15/2026**

Produced by **CW Data Science & Applied AI - HH Team** Model **cwhh-tif-60day-v1**

What CQI is: page 2. How to use this booklet: page 3.

CLINICAL QUALITY INTELLIGENCE

What is Clinical Quality Intelligence?

Purpose, how it works and what's in this booklet

CQI answers: Which patient needs care **today**, before it's too **late**? What **immediate actions** could a clinical manager take to **prevent** hospitalizations?

Most quality measures are **retrospective**: they report what happened after the episode ends. CQI is **proactive**: every week it connects risk, the care ordered and delivered, and the patient's trajectory, so the team can act **while the patient is still in care**.

Purpose: **Weekly Patient Conference Preparation | Built for the CWHH Clinical Quality team**

What CQI does

Compares the care each high-risk patient **needs** with the care they **received**, so clinicians can close gaps early.

What it reads

Visits, goals, vitals, OASIS, wounds, medications, events and notes. Co-created with CWHH Clinical Quality.

How AI is used

Predictive ML risk model on LighthouseAI; **rules** for care gaps; **LLMs** summarize, checked by LLM judges.

What's in this booklet

Care gaps
Missed, delayed or unscheduled visits
Pages 20-21 >

Risk drivers
Who is highest risk this week, and why
Page 9 >

Clinical picture
Diagnoses, wounds and OASIS
Pages 17-19 >

Vitals trajectory
Trend, urgency and action window
Page 10 >

Patient goals
Met, not met, therapy goals
Page 11 >

Medication adherence
High-risk meds, understanding, teaching
Page 12 >

Care plan gaps
Interventions, assessments, visit goals to add
Pages 13, 23 >

Discharge & network
Recommendation, PCP, CW PCO, CW Rx
Page 14 >

Where CQI is going

Clinicians stay in charge of every decision.

TODAY | PROTOTYPE

A weekly booklet and Power BI report built from operational data, reviewed by the clinical team in the patient conference. Clinician feedback shapes what comes next.

IN PRODUCTION | THE VISION

An **AI-assisted agent** that scans operational data **24/7 on clinicians' behalf**, surfaces care gaps and opportunities to improve outcomes **as they emerge**, and brings the right patient to the right clinician while there is still time to act.

CLINICAL QUALITY INTELLIGENCE

Inside this booklet

How to use it in the weekly patient conference, and where to find each section

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

How to use this booklet in conference

1. **Start with At a glance (page 4), then the one-page summary (page 5).**
   Why the patient is on the list, four status tiles and the recommended actions.
2. **Review the CQI analysis (pages 6-14).**
   What CQI found, concluded and recommends: CQI summaries, risk, vitals, goals, medications, care plan gaps.
3. **Check it against the evidence (pages 15-24).**
   The operational records behind each finding. Each page ends with a feedback tip.
4. **Give your input on screen (pages 25-27).**
   Rate each finding, record the conference decision, save and return.

Contents

CQI analysis pages show what CQI worked out: predicted risk, rule-based checks and AI-written summaries. **Evidence** pages show the operational records behind them, so you can check CQI.

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CQI analysis

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What CQI found, concluded and recommends

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Evidence

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The records behind the findings, and how CQI used them

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Your input

Section starts p. 25

Fillable on screen: feedback, decision, notes

Clinician feedback	26
Conference decision and notes	27

Colour key

- Act now / urgent, missed, or a data gap to resolve
- In progress / this week
- Monitor
- On track / met

At a glance

From the patient's stated goal to what needs attention now

From stated goal to what needs attention now

Patient goal	*“I want to stay at home and live as long as I can.”*
Risk	Readmission after HF hospitalization; AFib on anticoagulant, recent fall, lives alone with impaired vision.
Care delivery	Acuity window met. SN short in weeks 3 and 5, PT in weeks 4-5; OT evaluation and MSW consult not done.
Clinical trajectory	Weight up 6.3 lb over 3 weeks with new ankle edema; SpO2 drifting 95% to 91%. No vitals since DAY+33.
CQI flags	**Call the patient and notify MD of the weight gain today. Schedule all remaining SN and PT visits and escalate to the clinical manager.**

ACTION WINDOW	VITALS TRAJECTORY	DATA GAPS	DISCHARGE
Clinician review within 24 hours	Deteriorating	6	Not due

Care network for discharge: PCP on record | CW PCO: Refer | CW Rx: Refer

FEEDBACK TIP Before rating **At a glance + CQI flags** on **page 26** (fillable), check: the goal is the patient's own wording; each row matches the evidence pages; the CQI flags are the right next steps.

CWHH | WEEKLY PATIENT CONFERENCE

Clinical Quality Intelligence

Patient conference summary

Proof of concept: High-Risk & Rising-Risk Patients

cwhh-tif-60day-v1 | CQI run 08/15/2026 | Week of 08/17/2026

Patient ID	9000123
Episode	7000456
Branch	Branch 0412
Status	CURRENT
Admission	Readmission
Episode day	36 of 60

60-DAY HOSPITALIZATION RISK	PERCENTILE	TREND	CARE GAPS RAISE RISK	DELAY IMPACT
High 0.38	Top 34%	Rising +0.07 vs. prior run	Yes	Moderate

Why this patient is on today's list

High and rising hospitalization risk, weight up 6.3 lb in 3 weeks, and no future visits scheduled for the 7 ordered visits that remain.

IN THE PATIENT'S WORDS

"I want to stay at home and live as long as I can."

Care delivery gaps — ACT NOW

- 7 ordered visits remain (4 SN, 3 PT); 0 scheduled
- 2 SN and 1 PT visits missed in weeks 3-5
- OT evaluation and MSW consult not completed

Summary p.8 | Evidence p.20 >

Clinical risk drivers — ACT NOW

- 60-day risk 0.38; rising +0.07 vs. prior week
- Top drivers: recent inpatient stay, dyspnea, HF
- Care gaps flagged as raising risk

Summary p.7 | Evidence p.9 >

Vitals trajectory — ACT NOW

- Weight +6.3 lb over 3 weeks (168.0 to 174.3)
- SpO2 drifting down 95% to 91%; pulse up to 102
- No vitals since day 33; no SN visit scheduled

Summary p.7 | Evidence p.10 >

Patient goals — MONITOR

- 6 care-plan goals: 1 achieved, 2 in progress
- 2 delayed (daily weights, safe transfers)
- 1 missed (MSW consult within 3 weeks)

Summary p.7 | Evidence p.11 >

Recommended actions for discussion

6 DATA GAPS - CHECK PAGE 8 FIRST

#	Action	Owner	When	Page
1	Call patient and notify MD of weight gain; review diuretic order and HF zone plan	SN / CM	Within 24 hrs	10
2	Schedule all remaining ordered visits (SN wks 6-9, PT wks 6-8)	Scheduling	Within 24 hrs	20
3	Obtain order and complete OT home-safety evaluation; MSW consult for living-alone support	CM / OT / MSW	This week	11
4	Anticoagulant-focused med reconciliation with large-print list and teach-back	SN	Next SN visit	11

Record the conference decision on page 27 and rate findings on page 26 (fillable) >

SECTION 1

CQI analysis

What CQI found, concluded and recommends

Pages 6-14

Everything in this section was **produced by CQI**: a predictive risk model, rule-based checks across visits, vitals, goals and medications, and AI-written summaries checked by LLM judges. This is what operational reports do not do: CQI **connects** the data, **explains** why the patient is at risk, and **recommends** what to do next.

Treat every finding as a draft for clinical review.

In this section

CQI summaries: profile, risk, vitals — AI-generated, unedited — 7

CQI summaries: care, missed care, data gaps — AI-generated, unedited — 8

Risk drivers and episode timeline — Why the model flags this patient — 9

Vitals trajectory and action window — Trend, urgency and when to act — 10

Goals met and not met — Including therapy goals — 11

Medication adherence — High-risk meds, understanding, teaching — 12

Care plan gaps: what to add — Interventions, assessments, visit goals — 13

Care network and referrals — PCP, CW PCO, CW Rx at discharge — 14

CQI ANALYSIS

CQI summaries: profile, risk and vitals

AI-generated text, printed unedited. Check each summary against its evidence page.

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

Visits completed	Scheduled visits	Missed care	IP events	Vitals days	Wound patient
18	4	3	1	10	No

Schedule status	Risk level	Frequency status	Care gaps raise risk	Delay impact	Acuity window missed	Episode status	Discharge decision
no future schedule	high	minor shortfall	TRUE	moderate	FALSE	CURRENT	No

Patient goal (care plan)

PATIENT WILL VERBALIZE AND DEMONSTRATE DAILY WEIGHT MONITORING AND 3 SIGNS OF HEART FAILURE EXACERBATION AND WHEN TO CALL THE AGENCY WITHIN 2 WEEKS

Patient Profile & Risk Summary

Evidence: pages 16-9

82-year-old woman readmitted after a heart failure hospitalization that ended 2 days before start of care, with atrial fibrillation on an anticoagulant, type 2 diabetes, CKD stage 3, impaired vision and a fall in the past 3 months. She lives alone; her daughter visits weekly. SN fell short in weeks 3 and 5 and PT in weeks 4-5, and no SN or PT visits are scheduled for the rest of the period. Risk Rationale: Baseline risk is high due to the recent HF hospitalization, exertional dyspnea, polypharmacy with an anticoagulant and fall history in a patient who lives alone. Care gaps - missed SN visits during a period of weight gain and zero future skilled visits - raise hospitalization risk by reducing clinical surveillance of fluid status and medication safety.

Vitals Trajectory Summary

Evidence: page 10

Ten vitals days in 33 days. No readings since DAY+33 and no SN visit scheduled. Weight alert on DAY+29 has no documented physician notification. CQI recommends a same-day phone check and MD notification.

FEEDBACK TIP Before rating **Profile & Risk Summary**, **Vitals + action window** on **page 26** (fillable), check: the profile, diagnoses and living situation match the chart (page 16); the trends described match the vitals charts (page 10).

CQI ANALYSIS

CQI summaries: care delivery and data gaps

Care delivery, missed care, delay impact and data gaps, printed unedited.

Care Delivery Summary

Evidence: page 20

Week 1 front-loading was met for SN and PT. SN fell short in weeks 3 and 5 and PT in weeks 4-5; neither shortfall was made up. The OT evaluation ordered for week 2 and the PRN MSW consult have no visit record. Schedule: Seven ordered visits remain in weeks 6-9 (4 SN, 3 PT) and none are scheduled; only HHA visits are on the calendar.

Missed Care Summary

Evidence: page 21

Missed Care Impact: Two SN visits (DAY+17 patient not home; DAY+31 patient requested reschedule) and one PT visit (DAY+26 staff availability) were missed and not made up. The DAY+31 SN miss came two days after a weight alert with no documented physician notification, so a key chance to assess fluid status was lost. The OT evaluation and PRN MSW consult have no visit record. For a patient who lives alone with impaired vision, these gaps matter more than the visit count suggests.

Clinical Delay Impact Explanation

Evidence: page 20

ACUITYSTATUS: Acuity level 2 requires the first visit within 48 hours; the first SN visit occurred on DAY+1, within the window. Delay impact is rated moderate because later SN gaps coincided with a rising weight trend rather than because of the start-of-care timing.

Data Gaps Found

6 TO RESOLVE

Missing or conflicting data CQI noticed - confirm in the chart first.

- OT evaluation ordered for week 2 has no visit, missed-visit record or cancellation.
- Weight alert on DAY+29 has no physician-notification record and no follow-up note.
- Pulse oximeter is listed as a needed supply but no delivery or in-home documentation was found.
- Edema noted on DAY+33 has no measurement or comparison to baseline.
- Living situation says the daughter visits weekly but no caregiver contact or availability is recorded.
- Apixaban teaching is marked partial on DAY+15 with no later update.

FEEDBACK TIP Before rating **Care Delivery Summary**, **Missed Care Summary**, **Delay Impact Explanation**, **Data Gaps Found** on **page 26** (fillable), check: visit counts match the weekly grid (page 20); missed visits and reasons match the log (page 21); each data gap is real and not already resolved in the chart.

CQI ANALYSIS

Episode timeline and risk drivers

Where the patient is in the episode, and why the model flags them

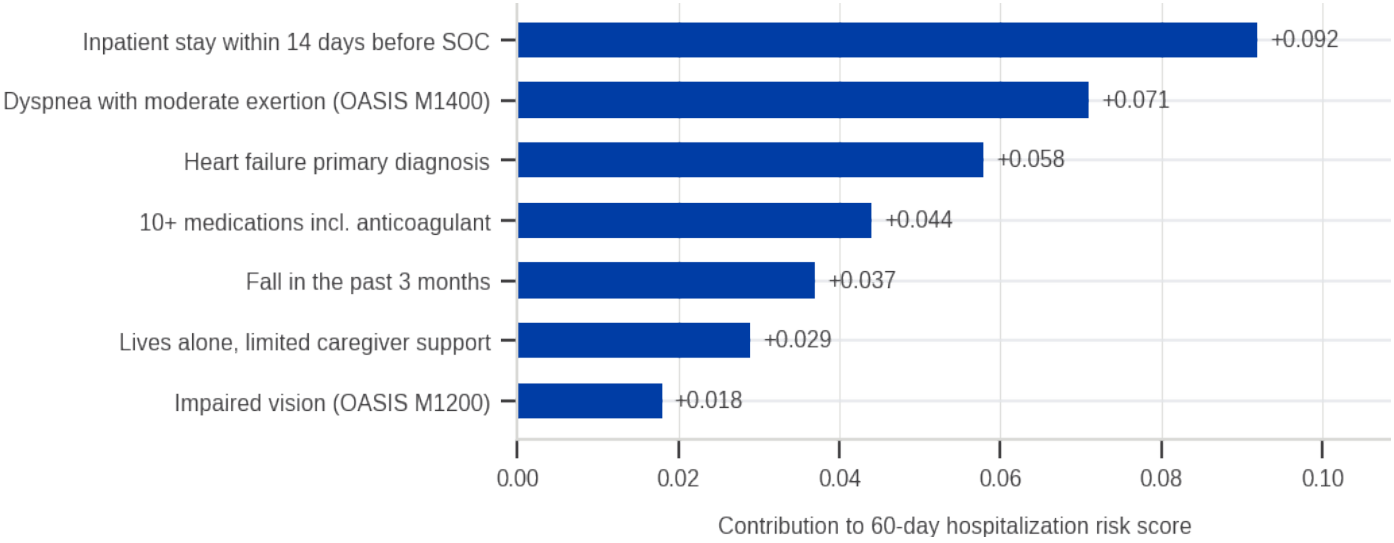
Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

Evidence for the CQI Patient Profile & Risk Summary and risk tile on page 7.

Episode timeline



Top drivers of 60-day hospitalization risk



FEEDBACK TIP Before rating **Risk tier and drivers** on **page 26** (fillable), check: the top drivers are clinically plausible for this patient; the prior inpatient stay and OASIS items match the chart.

CQI ANALYSIS

Vitals trajectory: action window

Weekly readings turned into a trajectory, an urgency and an action window

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

Evidence for the CQI Vitals Trajectory Summary on page 7.

WEEKLY VITALS
10 vitals days, last DAY+33

TRAJECTORY
Deteriorating

URGENCY
Within 24 hrs

ACTION WINDOW
Clinician review within 24 hours

What CQI saw

Weight up 6.3 lb over 3 weeks; SpO2 95% to 91%; pulse crossed 100 on DAY+29; no readings since DAY+33.

What CQI flags

Possible fluid retention. Clinician review within 24 hours; no SN visit scheduled is inadequate monitoring.

VITALS DAYS	DAYS SINCE LAST READING	NEEDING REVIEW	TRAJECTORY
10	3	3	Deteriorating

What CQI found, by measure

Measure	First	Latest	Change	CQI interpretation	Urgency
SpO2 (%)	95	91	**-4 in 5 wks**	Gradual decline, still above limit; consistent with weight trend	**Within 24 hrs**
Weight (lb)	168	174.3	**+6.3 in 5 wks**	Steady gain with edema noted DAY+33; possible fluid retention	**Within 24 hrs**
Systolic BP (mmHg)	132	152	**+20 in 5 wks**	Rising but below limit; recheck with HF review	**This week**
Pulse (bpm)	82	98	**+16 in 5 wks**	Crossed 100 on DAY+29; AFib - confirm apical rate and rhythm	**Routine**

FEEDBACK TIP Before rating **Vitals + action window** on **page 26** (fillable), check: readings match the visit documentation (units, dates, values); the trajectory and action window are clinically appropriate; the trend charts on page 22 support what CQI saw.

CQI ANALYSIS

Patient goals

Goals met, goals not yet met, and therapy goals

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

PATIENT-STATED GOAL (care plan wording, unchanged)

"I want to stay at home and live as long as I can."

GOALS MET	NOT YET MET	DELAYED / MISSED	THERAPY GOALS (PT/OT/ST)
2	6	3	4

Goals not yet met

Goal	Disc.	CQI status	Evidence found
MSW consult for social and community resources within 3 weeks	MSW	Missed	No MSW visit record
Weigh daily and keep a log	SN	Delayed	Log incomplete on 3 of 5 SN visits
Safe transfers with walker within 4 weeks	PT	Delayed	PT shortfall weeks 4-5
Verbalize anticoagulant bleeding precautions within 3 weeks	SN	In progress	Partially documented DAY+15
Home-safety evaluation completed	OT	In progress	OT evaluation not yet delivered
Independent with lower-body dressing using adaptive equipment	OT	Not started	OT evaluation not delivered

Goals met

Goal	Disc.	CQI status	Evidence found
Verbalize 3 signs of HF exacerbation within 2 weeks	SN	Achieved	Teach-back documented DAY+10
Ambulate 150 ft with walker, supervision, within 4 weeks	PT	Achieved	150 ft with walker DAY+24

Therapy goals (PT / OT / ST)

Goal	Disc.	CQI status	Evidence found
Safe transfers with walker within 4 weeks	PT	Delayed	PT shortfall weeks 4-5
Home-safety evaluation completed	OT	In progress	OT evaluation not yet delivered
Ambulate 150 ft with walker, supervision, within 4 weeks	PT	Achieved	150 ft with walker DAY+24
Independent with lower-body dressing using adaptive equipment	OT	Not started	OT evaluation not delivered

FEEDBACK TIP Before rating **Goals met / not met** on **page 26** (fillable), check: goal wording and status match the care plan; 'met' goals have documented evidence; therapy goals match PT / OT / ST plans.

CQI ANALYSIS

Medication adherence

Does the patient understand, have and take their medications as prescribed?

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

CQI summary

11 active medications including two high-risk: apixaban (bleeding risk) and insulin glargine (hypoglycemia). Understanding checked on 2 of 7 SN visits, last DAY+10. Missed doses reported DAY+15 and DAY+22; impaired vision makes labels hard to read.

ACTIVE MEDS	HIGH-RISK MEDS	UNDERSTANDING	ADHERENCE ISSUES	DAYS SINCE CHECK
11	2	2 of 7 visits	2	26

Flag: Adherence concern documented on 2 visit(s). Check understanding with teach-back and confirm the patient is taking each medication as prescribed.

High-risk medications

Medication	Class	Why it matters
Apixaban 5 mg BID	Anticoagulant	Anticoagulant: needs teaching and monitoring
Insulin glargine 20 units nightly	Insulin	Insulin: needs teaching and monitoring

Understanding, teaching and adherence at visits

Visit	Discipline	Understanding assessed	Teaching done	Taking as prescribed
DAY+1	SN	Yes	Yes	Yes
DAY+3	SN	No	No	-
DAY+5	SN	No	No	-
DAY+8	SN	No	Yes	Yes
DAY+10	SN	Yes	Yes	Yes
DAY+15	SN	No	Yes	No
DAY+22	SN	No	No	No

FEEDBACK TIP Before rating **Medication adherence** on **page 26** (fillable), check: the high-risk list matches the medication profile; understanding, teaching and adherence are documented on the visits shown; adherence barriers (vision, cost, cognition, supply) are addressed in the plan.

CQI ANALYSIS

Care plan gaps: what to add

Interventions missing per goal, assessments or vitals to add, and visit goals to add

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

Interventions missing per goal 4

Care-plan goal	Missing intervention	Disc.
Weigh daily and keep a log	**Large-print weight log and daily reminder call; escalate gain >3 lb/2 days to MD**	**SN**
Safe transfers with walker within 4 weeks	**Home exercise program for sit-to-stand with written large-print copy**	**PT**
Home-safety evaluation completed	**OT home-safety evaluation (grab bars, rugs, lighting)**	**OT**
MSW consult for social and community resources within 3 weeks	**MSW visit for meal delivery and personal emergency response system**	**MSW**

Assessments or vitals to add 4

Assessment / vital	Type	Why it is needed
Orthostatic blood pressure	Vitals	Near-fall on DAY+19 while on diuretic
Repeat fall-risk (MAHC-10)	Assessment	Last done at SOC; near-fall since
Lung sounds / edema grading each SN visit	Assessment	Weight trend up; edema noted DAY+33 without grade
Daily home weights reported by phone	Vitals	No SN visit scheduled; weight is the earliest HF warning sign

Visit goals to add to meet patient goals 4

Disc.	Visit goal	Supports patient goal
SN	Patient states HF zones and when to call using the large-print zone sheet	Stay at home; HF self-management
SN	Weight log reviewed every visit; MD notified for gain >3 lb/2 days	Weigh daily and keep a log
PT	Sit-to-stand x5 with walker without loss of balance	Safe transfers with walker
MSW	Community resources and emergency response set up	Stay at home living alone

Medications, supplies and equipment linked to goals 5

Item	Type	Supports goal	CQI alignment check
Furosemide 40 mg daily	Medication	Daily weights / HF goal	**Order aligned; weight trend not escalated**
Apixaban 5 mg BID	Medication	Bleeding precautions	Teaching in progress
Digital scale	Supply	Daily weights	Present, documented DAY+1

+ 2 more items not shown - see the CQI Power BI report.

FEEDBACK TIP Before rating **Care plan gaps to add**, **Medications and supplies** on **page 26** (fillable), check: each missing item is truly missing (not just undocumented); suggested visit goals are measurable and fit the patient's goal; medications and supplies match the current med profile.

Care network and discharge referrals

Primary care, CenterWell Primary Care (CW PCO) and CenterWell Pharmacy (CW Rx)

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

Refer to CenterWell Primary Care (CW PCO)?

Refer

CW PCO clinic 4.2 mi away; clinic close by.

Refer to CenterWell Pharmacy (CW Rx)?

Refer

11 active medications; home delivery and pharmacist review support adherence.

Care network

Item	Status	Detail
Primary care provider (PCP)	**Yes**	Community Internal Medicine
Last PCP visit	**166 days ago**	
Enrolled in CW PCO	**No**	
Enrolled in CW Rx	**No**	
Plan eligible (MA)	**Yes**	
Nearest CW PCO clinic	**4.2 mi**	CenterWell Senior Primary Care - Hickory
CW PCO clinic within 10 miles	**Yes**	2 clinic(s)
CW PCO clinic within 20 miles	**Yes**	3 clinic(s)
Active medications	**11**	

How the referral indicator is set

- **CW PCO:** Refer if not enrolled, plan-eligible and a clinic is within 10 miles (stronger when there is no PCP). Consider if the nearest clinic is 10-20 miles away. Not indicated if none within 20 miles or not eligible.
- **CW Rx:** Refer if not enrolled, plan-eligible and taking 5 or more active medications. Already enrolled: share the discharge medication list.
- **Check eligibility** appears when plan eligibility is unknown. Indicators are prompts for the discharge conversation, not orders.

FEEDBACK TIP Before rating **Care network + referrals** on **page 26** (fillable), check: PCP and enrollment match the member record; the referral indicator makes sense for this patient; the patient or caregiver agrees before any referral.

SECTION 2

Evidence

The records behind the findings, and how CQI used them

Pages 15-24

These pages show the **operational records** CQI used, organized for this patient: the same data you would find in the EMR and standard reports. Use them to **confirm or challenge** the findings in CQI analysis. The last page explains **how CQI produced** those findings: methods, data sources and known limits.

Each page ends with a feedback tip that links back to the finding it supports.

In this section

Patient context and 485 profile — Demographics, living situation, profile — 16

Diagnoses and procedures — Codes, onset and exacerbation — 17

Wounds — Type, size over time, healing — 18

OASIS — Start of care vs. latest — 19

Visits: ordered, delivered, scheduled — Weekly grid by discipline — 20

Missed care and events — Missed visits, alerts, logs, inpatient — 21

Vitals trends — Charts against patient limits — 22

Assessments, interventions, adverse events — What was done at visits — 23

How CQI produced this — Methods, data sources, limits — 24

EVIDENCE

Patient context

Who the patient is: demographics, diagnoses, living situation and 485 / OASIS profile

Evidence for the CQI Patient Profile & Risk Summary on page 7.

Patient context

Age band / sex	80-84 / F	**Clinical group**	MMTA_CARDIAC
Primary diagnosis	Heart failure, unspecified (I50.9)	**Comorbidities**	Type 2 diabetes; CKD stage 3; atrial fibrillation on anticoagulant; impaired vision
Living situation	Lives alone; daughter visits weekly	**Admission type**	Readmission
Acuity level / first visit	Level 2; first visit DAY+1		

Clinical profile (485 / OASIS)

Comorbidity adjustment	High	**Functional impairment**	Medium
Prognosis	Fair	**Rehab potential**	Fair
Functional limitations	Ambulation, endurance, dyspnea with minimal exertion, decreased vision	**Activities permitted**	Up as tolerated, walker, transfer bed/chair
Mental status	Oriented, forgetful	**Allergies**	PENICILLIN, SULFA
Nutritional requirements	2 gm sodium, 1500 ml fluid restriction		

FEEDBACK TIP Before rating **Profile & Risk Summary** on **page 26** (fillable), check: diagnoses, comorbidities and living situation match the chart; allergies, mental status and activities permitted are current on the 485.

EVIDENCE

Diagnoses and procedures

Primary and secondary diagnoses, onset or exacerbation, and procedures this episode

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

CQI summary

Primary HF (I50.9) with exacerbation onset before SOC. Comorbid AFib on apixaban, type 2 diabetes and CKD stage 3. No procedure codes this episode.

DIAGNOSES	PROCEDURES	ONSET / EXACERBATION	BODY SYSTEMS
6	0	3	5

Diagnoses

Code	Description	Body system	Clinical group	Onset / exacerbation
I50.9	**Primary:** Heart failure, unspecified	Circulatory System	MMTA_CARDIAC	**Exacerbation (DAY-6)**
I48.91	Atrial fibrillation, unspecified	Circulatory System	MMTA_CARDIAC	**Onset (DAY-400)**
E11.9	Type 2 diabetes without complications	Endocrine	MMTA_ENDO	
N18.30	Chronic kidney disease, stage 3 unspecified	Genitourinary	MMTA_OTHER	
L89.152	Pressure ulcer of sacral region, stage 2	Skin	WOUND	**Onset (DAY-3)**
Z79.01	Long term use of anticoagulants	Factors Influencing Health		

Procedures

Code	Description	Clinical group	Date
No procedures recorded this episode			

FEEDBACK TIP Before rating **Diagnoses, wounds, OASIS** on **page 26** (fillable), check: diagnosis codes and the primary diagnosis match the 485 / plan of care; onset vs. exacerbation is coded correctly; the procedures listed belong to this episode.

EVIDENCE

Wounds

Wound type, location, size over time and healing status

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

CQI summary

One stage 2 sacral pressure injury, area down 33% since SOC and assessed weekly. A skin tear on the left forearm from the DAY+19 near-fall has no follow-up measurement.

WOUNDS	PRESSURE ULCERS	HEALED	NOT MEASURED IN 7+ DAYS
2	1	0	2

Wound inventory

Location	Type	Stage	Size: first → latest	Change	Status	Count / last
SACRAL, MID	Pressure Ulcer	Stage 2	3x2x0.3 cm → 2.5x1.6x0.2 cm	**-33% area**	**Improving**	4 / D+22
FOREARM, LT	Trauma Skin Tear	-	2x0.5 cm → 2x0.5 cm	**-**	**Not reassessed**	1 / D+19

Wound descriptions

- **SACRAL, MID:** Stage 2 pressure ulcer - partial thickness skin loss
- **FOREARM, LT:** Skin tear from near-fall

FEEDBACK TIP Before rating **Diagnoses, wounds, OASIS** on **page 26** (fillable), check: wound type, location and stage match the wound assessment; measurements and healing status are current; every open wound has a treatment order.

EVIDENCE

OASIS

Key OASIS items at start of care and the latest assessment, with changes highlighted

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

CQI summary

SOC OASIS shows dyspnea with moderate exertion, ambulation with a device, and 5+ medications. Fall-risk and caregiver items (M2102) suggest the patient needs supervision support that is not in the plan.

ITEMS CAPTURED	KEY RISK ITEMS	CHANGED SINCE SOC	ASSESSMENTS
11	10	0	1

OASIS items (key risk items first)

Item	Section	Description	Answer
M1033	Risk for hospitalization	Risk for hospitalization	1 - History of falls; 3 - 2+ hospitalizations in 6 months; 5 - Currently taking 5+ medications
M1400	Respiratory status	When is the patient dyspneic	2 - With moderate exertion
M1850	ADL / IADL	Transferring	1 - Transfers with minimal human assistance or device
M1860	ADL / IADL	Ambulation / locomotion	2 - Requires use of a two-handed device (walker)
M2020	Medications	Management of oral medications	2 - Able if another person prepares
M1242	Pain	Frequency of pain interfering with activity	1 - Less often than daily
M1700	Neuro / cognitive	Cognitive functioning	1 - Requires prompting under stressful conditions
M1100	Living arrangements	Patient living situation	Lives alone; occasional assistance
M2102	Care management	Supervision and safety	2 - Non-agency caregiver(s) need training / supportive services
M1311	Integumentary	Number of stage 2 pressure ulcers	1
M1200	Sensory	Vision	1 - Partially impaired

Assessments: 01 - Start of care.

FEEDBACK TIP Before rating **Diagnoses, wounds, OASIS** on **page 26** (fillable), check: answers match the OASIS in the EMR; changed items reflect a real change in the patient; M1033 risk factors are addressed in the care plan.

EVIDENCE

Care delivery

Ordered vs. delivered vs. scheduled visits by discipline and week

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Evidence for the CQI Care Delivery Summary and Delay Impact on page 8.

Weekly visit frequency

Discipline	Wk 1	Wk 2	Wk 3	Wk 4	Wk 5	Wk 6	Wk 7	Wk 8	Wk 9	Total	Left / sched.
SN	3/3	2/2	1/2	1/1	0/1	0 of 1	0 of 1	0 of 1	0 of 1	7/9	4 / 0
PT	1/1	2/2	2/2	1/2	1/2	0 of 1	0 of 1	0 of 1	-	7/9	3 / 0
OT	-	0/1	-	-	-	-	-	-	-	0/1	0 / 0
HHA	-	1/1	1/1	1/1	1/1	1 of 1	1 of 1	1 of 1	1 of 1	4/4	4 / 4
MSW	-	-	0/1	-	-	-	-	-	-	0/1	0 / 0

Weeks 1-5: delivered/ordered (green = met, plum = short). Weeks 6-9: scheduled of ordered.
Light blue rule = CQI run date. Left / sched. = ordered visits remaining vs. visits on the calendar.

FREQUENCY STATUS	SCHEDULE STATUS	ACUITY WINDOW MISSED	MISSED-CARE RECORDS
Minor shortfall	No future schedule	No	3

FEEDBACK TIP Before rating **Care Delivery Summary**, **Delay Impact Explanation** on **page 26** (fillable), check: ordered frequencies match the 485 / plan of care; delivered counts match the visit record; the future schedule is confirmed in the scheduling system today.

EVIDENCE

Missed care and events

Missed or outstanding visits, inpatient events, vitals alerts and client log notes

Evidence for the CQI Missed Care Summary on page 8.

Missed and outstanding care

When	Discipline	Visit	Reason / evidence	Status
DAY+17	**SN**	SN subsequent visit	Patient not home	**Missed**
DAY+26	**PT**	PT subsequent visit	Staff availability	**Missed**
DAY+31	**SN**	SN subsequent visit	Patient requested reschedule	**Missed**

Inpatient events, vitals alerts and client log notes

When	Type	Detail	Source / follow-up
DAY-6	**Inpatient event**	HF exacerbation; discharged 2 days before SOC	Source: inpatient events
DAY+17	**Client log note**	Patient not home for SN visit; voicemail left	Source: client logs
DAY+29	**Vitals alert**	Weight above parameter (+5.4 lb from baseline)	**No physician notification found**
DAY+29	**Vitals alert**	Pulse 102 above parameter (100)	Physician notified
DAY+31	**Client log note**	Patient requested reschedule; not rebooked	Source: client logs

FEEDBACK TIP Before rating **Missed Care Summary** on **page 26** (fillable), check: missed visits and reasons match the visit record and client logs; alerts flagged in plum had no documented physician notification.

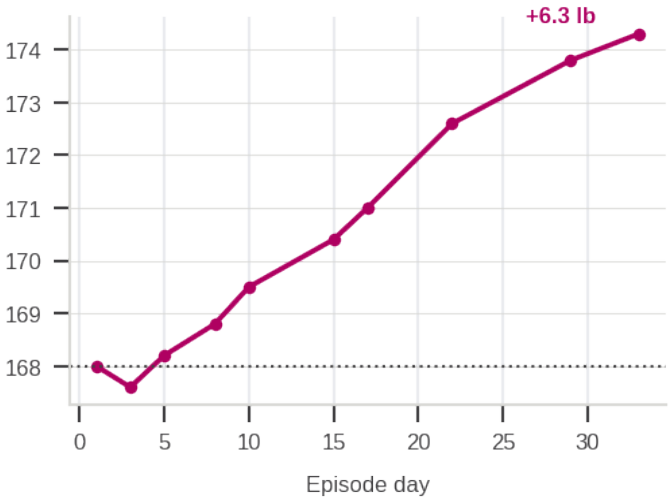
Vitals trends

Each measure against the patient's limits, by episode day

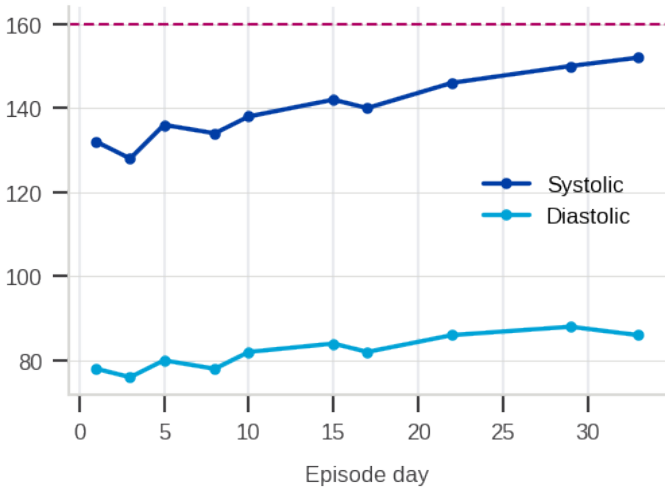
Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

Evidence for the CQI vitals action window on page 10.

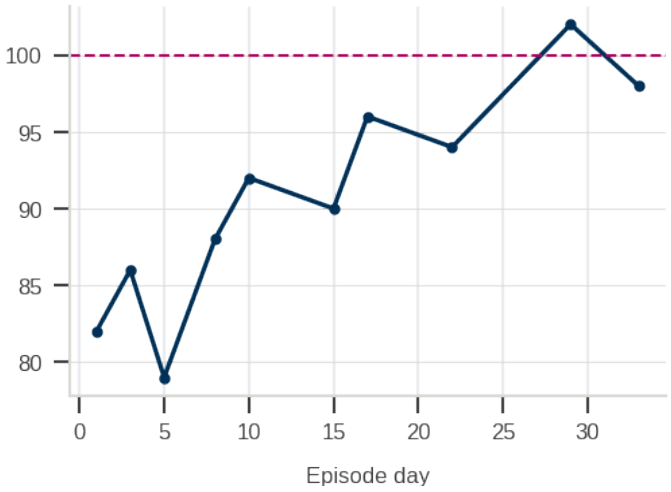
Weight (lb) - baseline dotted



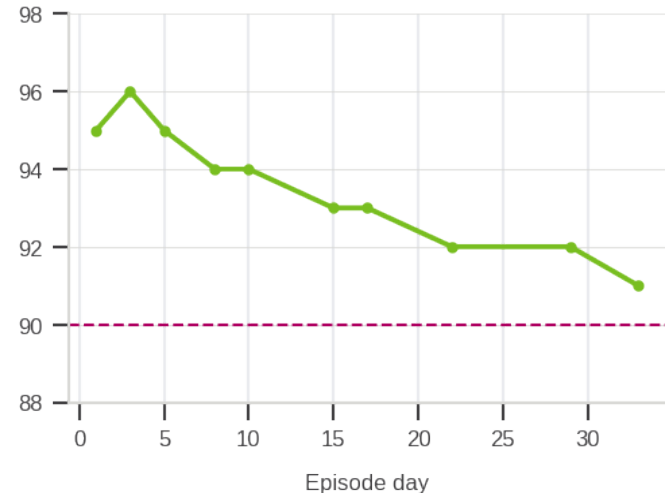
Blood pressure (mmHg) - limit SBP 160



Pulse (bpm) - limit 100



SpO2 (%) - limit 90



FEEDBACK TIP Before rating **Vitals + action window** on **page 26** (fillable), check: readings match the visit documentation (units, dates, values); limits match the patient's ordered call parameters.

EVIDENCE

Care delivered

Assessments and interventions performed, adverse events and visit narrative themes

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Assessments performed

SN completed cardiopulmonary and edema assessments at every SN visit and medication review twice. Fall-risk (MAHC-10) was done at SOC only. Pain and depression screens were done at SOC and not repeated. No home-safety assessment has been documented because the OT evaluation was not delivered.

Assessment	Disc.	Count	Last day
Cardiopulmonary assessment	SN	7	DAY+22
Edema assessment	SN	7	DAY+22
Gait and balance assessment	PT	3	DAY+29
Medication review	SN	2	DAY+15

+ 3 more assessments not shown - see the CQI Power BI report.

Interventions performed

Most interventions targeted HF self-management: HF teaching (5 visits), daily-weight teaching (3), medication teaching (3) and gait/transfer training by PT (6). Anticoagulant bleeding-precaution teaching was documented once. No intervention addressed the living-alone risk, and no diuretic or weight-escalation intervention was documented after the DAY+29 weight alert.

Intervention	Disc.	Linked care-plan goal	Count	Last day
Gait and transfer training	PT	Safe transfers with walker within 4 weeks	6	DAY+24
HF teaching	SN	Verbalize 3 signs of HF exacerbation within 2 weeks	5	DAY+10
Daily-weight teaching	SN	Weigh daily and keep a log	3	DAY+15
Medication teaching	SN	Not linked to a goal	3	DAY+22
Anticoagulant bleeding-precaution teaching	SN	Verbalize anticoagulant bleeding precautions within 3 weeks	1	DAY+15

Adverse event summary

One near-fall on DAY+19 (caught by daughter, no injury) and a weight alert on DAY+29 without documented physician notification. No ED visit or hospitalization since SOC.

DAY+19 Near-fall (No injury): Lost balance getting up from chair; caught by daughter

Visit narrative themes

- *"Shortness of breath when walking to the bathroom"*
- *"Forgets to weigh on days the daughter does not visit"*
- *"Worried about bleeding on apixaban"*
- *"Wants to stay at home and avoid another hospital stay"*
- *"Difficulty reading medication labels"*

FEEDBACK TIP Before rating **Assessments + interventions**, **Adverse events + themes** on **page 26** (fillable), check: counts match the visit documentation; the summaries describe what was actually done, not what was ordered; every adverse event is listed and correctly classified.

EVIDENCE

How CQI produced this summary

Methods, data sources and definitions: what to trust and what to check

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From data to findings

1 Predict

Production 60-day hospitalization risk model scores high and rising-risk patients each week and explains top drivers.

2 Compare

Rules compare ordered, scheduled and delivered visits by discipline and week, timing windows and vitals limits.

3 Read

Language models read goals, care plans, notes and logs to draft the summaries on pages 7-8.

4 Check

AI drafts are checked by a panel of LLM judges and adjudicated. Clinician feedback from this pilot will guide improvements.

Data sources

Home Health operational data on the CW Lighthouse AI environment: admissions and episodes, visits, 485 plan of care and orders, vitals, assessments, interventions, OASIS, inpatient events, events, client logs, goals, notes, diagnoses and procedures, referrals, PDGM periods, supplies, medications, care-plan tasks, wounds.

Model outputs: CWHH 60-day transfer-to-inpatient risk scores and risk drivers.

Definitions

High / rising risk	High: top 25% of weekly risk scores. Rising: weekly score increasing versus prior runs.
Frequency status	on_track, minor_shortfall, behind or significantly_behind versus ordered visits to date.
Schedule status	Whether remaining ordered visits are on the calendar (e.g. no_future_schedule, needs_change).
Risk raised by care gaps	CQI judgment that missed or delayed care materially increased risk.
Delay clinical impact	Estimated clinical impact of timing delays: low, moderate or high.
Nearing discharge decision	Active episode within 14 days of the episode end date.
Urgency	Within 24 hrs, this week, or next visit - a recommendation for the team, not an order.

Known limits

- Findings reflect data available at the run date; documentation lag can look like a care gap.
- Missing goals or vitals may be a documentation gap rather than a care gap - check the chart.
- Recommendations are drafts for clinical review.

Questions or corrections

CW Data Science & Applied AI - HH Team | For more information visit go/lighthouse

SECTION 3

Your input

Fillable on screen: feedback, decision, notes

Pages 25-27

Your review makes CQI better. Rate each finding, record the conference decision and add notes **on screen**, then save and return the PDF. Answers are collected automatically.

Open in Adobe Acrobat Reader, Microsoft Edge or Chrome to fill in.

In this section

Clinician feedback — Rate each finding — 26

Conference decision and notes — Decision, owner, follow-ups — 27

YOUR INPUT

Clinician feedback (fillable)

Complete on screen, save and return. One form per patient; answers are collected automatically.

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How to complete: open this PDF in Adobe Acrobat Reader, Microsoft Edge or Chrome, click the circles and boxes, type comments, then save (Ctrl+S) and return it to the CW Data Science & Applied AI - HH Team. **Tip:** rate at least the findings you discussed. For *Partly* or *Disagree*, tick an error type and add one line saying what was wrong - that is what improves CQI.

CQI finding	Pages	Agree	Partly	Disagree	Error type D I C N	Comment (what was wrong or missing)
At a glance + CQI flags	4	☐	☐	☐	☐ ☐ ☐ ☐	
Summary page + actions	5	☐	☐	☐	☐ ☐ ☐ ☐	
Profile & Risk Summary	7, 16	☐	☐	☐	☐ ☐ ☐ ☐	
Risk tier and drivers	9	☐	☐	☐	☐ ☐ ☐ ☐	
Vitals + action window	7, 10	☐	☐	☐	☐ ☐ ☐ ☐	
Care Delivery Summary	8, 20	☐	☐	☐	☐ ☐ ☐ ☐	
Missed Care Summary	8, 21	☐	☐	☐	☐ ☐ ☐ ☐	
Delay Impact Explanation	8, 20	☐	☐	☐	☐ ☐ ☐ ☐	
Data Gaps Found	8	☐	☐	☐	☐ ☐ ☐ ☐	
Goals met / not met	11	☐	☐	☐	☐ ☐ ☐ ☐	
Medications and supplies	13	☐	☐	☐	☐ ☐ ☐ ☐	
Assessments + interventions	23	☐	☐	☐	☐ ☐ ☐ ☐	
Adverse events + themes	23	☐	☐	☐	☐ ☐ ☐ ☐	
Care plan gaps to add	13	☐	☐	☐	☐ ☐ ☐ ☐	
Diagnoses, wounds, OASIS	17-19	☐	☐	☐	☐ ☐ ☐ ☐	
Medication adherence	12	☐	☐	☐	☐ ☐ ☐ ☐	
Care network + referrals	14	☐	☐	☐	☐ ☐ ☐ ☐	

Error type: D = data missing I = data incorrect C = clinical interpretation N = not relevant for this patient

Conference outcome (tick all that apply)

☐ Plan updated ☐ Visits rescheduled ☐ MD notified ☐ No change needed ☐ Refer to Data Science

How useful was this booklet for conference?

☐ 1 Not useful ☐ 2 ☐ 3 ☐ 4 ☐ 5 Very useful

What should CQI add, remove or change?

Reviewer name

Role / discipline

Date

YOUR INPUT

Conference decision and notes (fillable)

Record the team's decision here. Saved with the feedback form.

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

Conference decision

☐ Agree with CQI ☐ Agree with changes ☐ Disagree ☐ Escalate to MD

Action owner	Due date	Reviewed by

Discussion notes

Follow-ups and owners

Anything else for the CQI team